Adjuvant abemaciclib + endocrine therapy

> **Confirm the individual supportive prescription.** This review aid does not generate a medication order. Use the current NCCP regimen, antiemetic guidance and local pharmacy plan; assess interactions, allergies and comorbidities.

Antiemetic and premedication review

- Oral moderate–high category; individualise rescue/prevention under local policy
- Have the prescribed antidiarrhoeal medicine available. Start the agreed plan at the first loose stool and call early.
- Take the prescribed dose at regular times. Do not double up after a missed or vomited dose.

Additional supportive care

- Have the prescribed antidiarrhoeal medicine available. Start the agreed plan at the first loose stool and call early.
- Take the prescribed dose at regular times. Do not double up after a missed or vomited dose.
- Keep early blood-test appointments. Check every new medicine or supplement with oncology/pharmacy; avoid grapefruit.
- Confirm endocrine/OFS treatment, bone-health assessment and the longer-term follow-up plan.

Monitoring and handover

- Early blood monitoring: Baseline; every 2 weeks for 2 months; monthly for the next 2 months. After month 4: as clinically indicated, with additional tests for toxicity. Do not copy a ribociclib monitoring schedule.
- Early diarrhoea support: From the first loose stool and throughout treatment. Grade 2 that has not resolved within 24 h, persistent/recurrent grade 2, grade 3–4 or hospitalisation require the source interruption pathway. Do not automatically hold all first grade 2 episodes without duration/context.
- Bone density and fracture risk: At AI/OFS initiation and then by risk. Repeat DEXA is based on baseline bone density, fracture risk and bone treatment. A 12–24-month reassessment can be appropriate near a treatment threshold or when risk changes; it is not an automatic annual scan for everyone. Tamoxifen alone is not an automatic baseline DEXA indication. Adjuvant bisphosphonate eligibility is a separate decision.
- Breast surveillance: After early breast treatment. Routine CT, PET or tumour-marker blood tests are not recommended simply to look for recurrence after early breast treatment in someone without symptoms. Imaging is arranged for a clinical reason.

- Two-year transition: At planned abemaciclib completion. Do not infer that endocrine therapy ends when abemaciclib ends. Prior anthracycline/HER2 exposure may create a separate cardiac follow-up need.

Access, visits and patient preparation

- Interactions and storage: Check all new medicines and supplements with oncology/pharmacy, avoid grapefruit, store as labelled and keep away from children. Return unwanted tablets to the pharmacy.
- Endocrine and ovarian-suppression visits: An aromatase inhibitor is generally a daily tablet. Fulvestrant, when selected for metastatic treatment, has loading injections on Days 1, 15 and 29 then monthly. Ovarian-suppression injections have their own prescription and visit schedule.
- No infusion pump: The CDK4/6 medicine is taken orally. An implanted port or infusion pump is not required for these tablets; other treatment may have separate access needs.

Patient handover

Explain the current phase, how to take each medicine, which tests are expected next and who arranges and reviews them. Confirm urgent contact details. Provide the patient portal, visual guide, passport, treatment card and whole-treatment map.

Fever, shivers or suddenly feeling unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: emergency help, 112 / 999.

Sources

NCCP 00619 v4a · official regimen

EMA · Verzenios product information and patient leaflet

NICE NG101 · bone health and follow-up

ESC 2022 cardio-oncology guideline · cardiac surveillance and risk

NOGG · section 7, aromatase inhibitors and fracture risk

NCCP supportive-care antiemetics for NCIS

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